

Policy on Storage and Availability of Medical Supplies and Consumables

«Hospital Name»

Document control

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Date created	_____	Date of implementation	_____
Prepared by	Stores In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

Medical supplies and consumables are acquired through a defined process, stored safely and cleanly, controlled through sound inventory practice, and checked for condition before every use.

1. Purpose

This policy explains how «Hospital Name» acquires medical supplies and consumables, stores them safely, controls inventory soundly, and verifies their condition before they're used. In this policy, "medical supplies and consumables" means items used in patient care other than medications and implants — those are covered in other hospital policies.

This policy does not cover medication or implant storage in detail — those are covered in other hospital policies.

2. Scope

This policy applies to Stores staff and all clinical staff who acquire, store, or use medical supplies and consumables at «Hospital Name».

3. Policy standards

«Hospital Name» acquires medical supplies and consumables through a defined process, uses them safely with precautions to maintain sterility and integrity where appropriate,

stores them in a clean, safe, and secure environment following manufacturer recommendations, applies sound inventory control practices, and verifies their condition before dispensing and use.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

1. Do not acquire medical supplies or consumables outside the defined process covering vendor selection, vendor evaluation, indenting, purchase order generation, and receipt of goods.
2. Do not open or use a medical supply or consumable without the precautions needed to maintain its sterility and integrity, where appropriate.
3. Do not store medical supplies or consumables outside the manufacturer's recommendations, in any area including wards, and do not leave hazardous materials unidentified or unsafely stored.
4. Do not store medical supplies or consumables without following a sound inventory control practice.
5. Do not dispense or use a medical supply or consumable without first checking its condition.

5. What we do

5.1 Acquire supplies through a defined process

The acquisition process for medical supplies and consumables addresses five things: vendor selection, vendor evaluation, the indenting process, purchase order generation, and receipt of goods.

5.2 Use supplies safely

Where appropriate, medical supplies and consumables are opened and used with the precautions needed to maintain their sterility and integrity.

5.3 Store supplies safely

Storage requirements specified by the manufacturer are followed, in every area supplies are stored, including wards. Supplies are protected from loss or theft, storage areas are kept clean, and hazardous materials are identified and kept safely.

5.4 Apply sound inventory control

«Hospital Name» follows or demonstrates sound inventory control practices — for example ABC analysis, VED (vital/essential/desirable), FSN (fast/slow/non-moving), First-Expiry-First-Out, or lead-time analysis. These are examples; «Hospital Name» may use any combination of practices it can demonstrate is actually in use, not just documented.

5.5 Verify condition before use

Medical supplies and consumables are kept in a condition suitable for safe use, and that condition is checked before dispensing and use — for example checking for an opened package, a damp cotton roll, physical damage, or discolouration. These are examples of defects to check for, not an exhaustive list.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring this policy is resourced and implemented.
Stores In-Charge	Owns day-to-day implementation of this policy; manages the defined acquisition process; maintains inventory control; ensures storage conditions are met in the main stores area.
Clinical staff — ward and department heads	Ensure manufacturer storage requirements are followed in their areas, including wards; check condition of supplies before use; ensure safe use precautions are applied where appropriate.
Quality Coordinator	Audits this policy; holds training records and staff acknowledgements.

7. Quality monitoring

Monitoring area	What is monitored
Defined acquisition process — all five elements	Process in place covering vendor selection, vendor evaluation, indenting, purchase order generation, and receipt of goods; all five elements present and working.
Safe use — sterility and integrity precautions	Precautions applied when opening and using supplies where appropriate; no instances of inappropriate opening technique or sterility break.
Storage compliance — including wards	Manufacturer storage requirements followed in all storage areas, including wards; supplies protected from loss or theft; storage areas clean; hazardous materials identified and safely stored.
Inventory control	Sound inventory control practice demonstrated as actually in use — not just documented; method(s) employed consistent with those declared.
Condition check before use	Condition checked before dispensing and use; defective supplies (opened, damaged, discoloured, damp) identified and

	removed.
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8. Training and staff acknowledgement

All Stores staff and clinical staff who acquire, store, or use medical supplies and consumables shall be familiar with the defined acquisition process, safe-use precautions, storage requirements (including in wards), inventory control expectations, and condition-checking requirements in this policy.

I have read the Policy on Storage and Availability of Medical Supplies and Consumables of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to the Stores In-Charge, Stores staff, ward and department heads, and the Quality Coordinator.

10. Abbreviations

Abbreviation	Meaning
ABC	Activity-Based Classification — value analysis method for inventory
FEFO	First-Expiry-First-Out
FSN	Fast/Slow/Non-moving — inventory turnover classification
MOM	Management of Medication (NABH Hospitals chapter)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
VED	Vital/Essential/Desirable — inventory criticality classification

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
MOM.11.a	Commitment*	Sections 3 and 5.1 address the defined acquisition process, covering all five elements: vendor selection,

		vendor evaluation, the indenting process, purchase order generation, and receipt of goods. The Purpose section also clarifies that this policy covers items used in patient care other than medications and implants — those are covered elsewhere.
MOM.11.b	Commitment	Section 5.2 addresses the safe opening and use of medical supplies and consumables with precautions to maintain sterility and integrity, where appropriate.
MOM.11.c	Commitment	Section 5.3 addresses storage per manufacturer requirements in every storage area, including wards — the "including wards" scope is stated explicitly. Also addresses protection from loss or theft, clean storage areas, and identification and safe storage of hazardous materials.
MOM.11.d	Commitment	Section 5.4 addresses sound inventory control practice — demonstrated as actually in use. Examples are given (ABC, VED, FSN, FEFO, lead-time analysis) but no specific method is mandated; the requirement is demonstrable practice, not any particular system.
MOM.11.e	Commitment	Section 5.5 addresses verification of condition before dispensing and use — for example checking for an opened package, a damp cotton roll, physical damage, or discolouration. Examples are not an exhaustive list.

12. Required Records/Evidence Checklist

Defined acquisition process — MOM.11.a (Commitment*)

6. Documented acquisition process covering all five elements: vendor selection, vendor evaluation, indenting, purchase order generation, and receipt of goods.
7. Working evidence for each element — for example, approved vendor list, vendor-evaluation records, indent forms, purchase orders, and goods-receipt notes.
8. Confirmation that supplies are not acquired outside this defined process.

Safe use — MOM.11.b

9. Training records or SOPs confirming staff know the precautions for maintaining sterility and integrity of the supplies they use.
10. Audit or observation evidence that appropriate precautions are followed when opening and using supplies.

Storage compliance including wards — MOM.11.c

11. Manufacturer storage-requirement records on file for each major category of supply.
12. Audit evidence that storage requirements are followed in all areas, including wards — temperature, humidity, light, and packaging requirements where applicable.
13. Hazardous materials identified and safely stored; inventory of hazardous materials on file.

Inventory control — MOM.11.d

14. Declaration of the inventory control method(s) in use (from the documented practice, not just from the policy).
15. Working records demonstrating the declared method is actually applied — for example ABC classification outputs, VED analysis records, FEFO stock rotation records, or lead-time analysis sheets.

Condition check before use — MOM.11.e

16. SOP or checklist for condition inspection before dispensing and use, naming the defects to check for.
17. Audit evidence that condition checks happen before dispensing and use.
18. Records of defective items identified, removed, and replaced.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. MOM.11.

Disclaimer

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